Assessing male genitalia, anus, prostate, and rectum

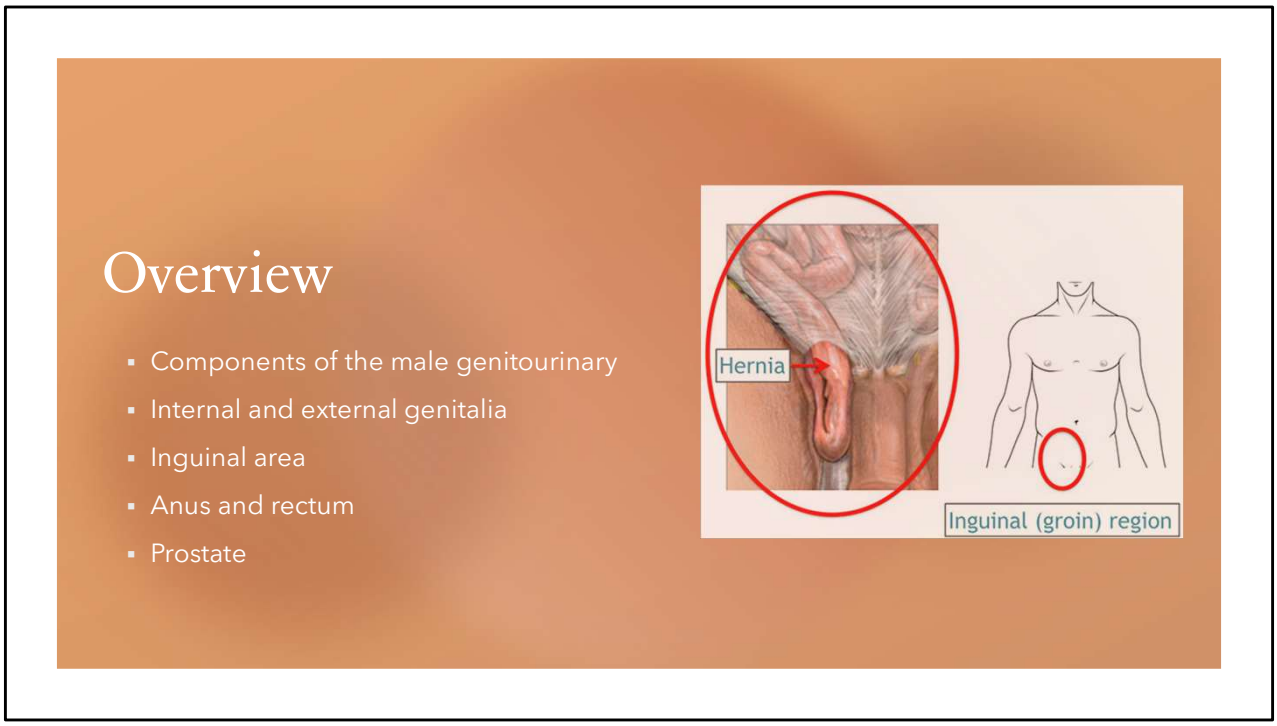
Chapter 26

Weber

NURS3120: Health Assessment and Promotion

Objectives

- 1. Describe the structure and function of the male genitalia, anus, rectum, and prostate
- 2. Discuss risk factors associated with prostate cancer and testicular cancer and ways to reduce one's risks
- 3. Differentiate between normal and abnormal findings of male genitalia, anus, rectum, and prostate
- 4. Analyze the client's interview and physical assessment data related to the male genitalia, anus, rectum, and prostate to make valid clinical judgments



The understanding helps us to guide the physical examination and readily assists the examiner in identifying abnormalities.

External Genitalia: Penis and the scrotum. The penis is the male reproductive organ. Penis may or may not have foreskin present (circumcision). The scrotum is a thin-walled sac that is suspended below the pubic bone, posterior to the penis. The scrotum functions as a protective covering for the testes, epididymis, and vas deferens and helps to maintain the cooler-than-body temperature necessary for production of sperm.

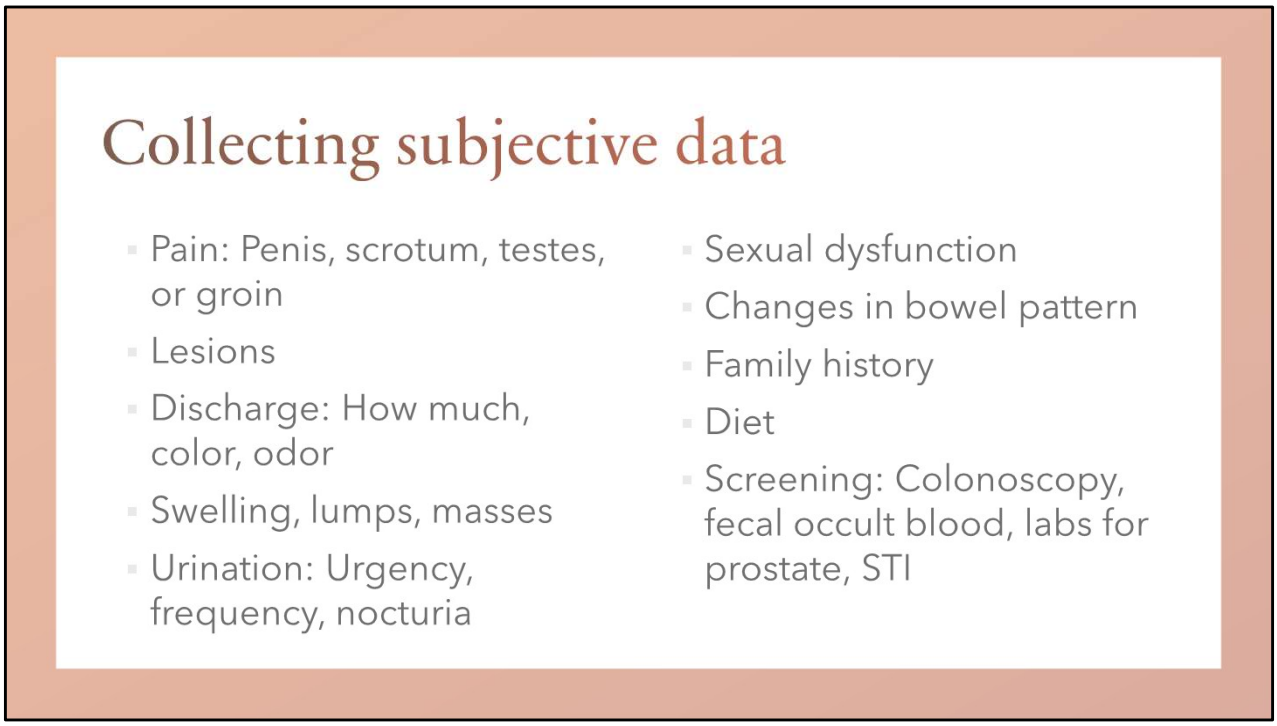
Internal: Testes, spermatic cord. Testes produce spermatozoa and the male sex hormone testosterone. The testes are suspended in the scrotum by a spermatic cord. The spermatic cord contains blood vessels, lymphatic vessels, nerves, and the vas deferens, which transports spermatozoa away from the testes.

Inguinal Area: inguinal or groin area because of hernias (protrusion of loops of bowel through weak areas of the musculature) are very common in this location.

Anus and Rectum: The anal canal is the final segment of the digestive system it begins at the anal sphincter and ends at the anorectal junction it is approx. 2.5-4cm long. The rectum is the lowest portion of the large intestine and is approximately 12cm long extending from the end of the sigmoid colon to the anorectal junction

Prostate: The prostate gland is approximately 2.5 to 4 cm in diameter, surrounding

the neck of the bladder and urethra; it lies between these structures and the rectum in male clients. It secretes a thin, milky substance that promotes sperm motility and neutralizes female acidic vaginal secretions. This chestnut- or heart-shaped organ can be palpated through the anterior wall of the rectum.

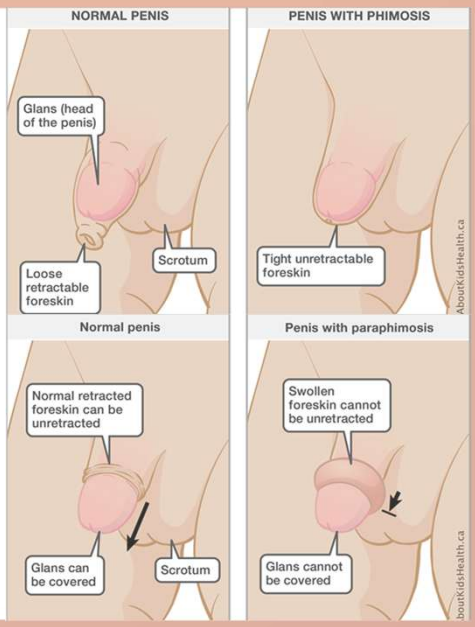


Pain: complaints of pain in these areas may indicate a hernia or an inflammatory process, epididymitis, herpes, prostatitis, STI, muscle strain, malignancy
Lesions: lesions may be a sign of a sexually transmitted infection or cancer
Discharge: may indicate infection
Swelling, lumps masses: may indicate infection, hernia, or cancer. Enlargement of the scrotum may indicate hydrocele, hematocele, hernia, or cancer.
Difficulty urinating: May indicate an infection or blockage, including prostate enlargement. Urinary frequency is usually caused by an overactive bladder by an enlarged prostate. Nocturia could be attributed to BPH, benign prostatic hyperplasia, dysuria is painful urination which is usually caused by an infection UTI
Color: Changes in the color, odor, or amount of urine could indicate an infection, blood in the urine (hematuria should be referred for medical investigation because this may indicate infection, kidney stones, benign prostatic hypertrophy or cancer. A decease in the amount of voided may indicate prostate enlargement or kidney problems or retention.
Sexual issues: Have you recently had a change in your pattern of sexual activity or sexual desire? decreased libido needs to be investigated to determine the cause. Trouble maintaining an erection or ED occurs frequently in adult males and may be attributed to various factors or disorders (alcohol use, diabetes, depression,

antihypertensive medications) Pain with ejaculation may indicate epididymitis. ED increases in frequency with age.
Bowel Pattern: A change in bowel pattern is associated with many disorders and is one of the warning signs of cancer.
Family history: Colorectal cancer risk is increased by a family history of colorectal cancer and/or polyps.
Diet: Fiber adds bulk to the digestive system and shortens the time that wastes, which often contain carcinogens, travel through the colon, thus decreasing the chance for intestinal cells to be affected. Red and processed meats are both implicated in cancer development, with processed meats being a “known” carcinogen to humans and red meat being a “probable” carcinogen. High-fat diets have been implicated in colon cancer.
Screening: Colonoscopy is recommended every 10 years. The American Cancer Society recommends a stool test every year to detect occult blood beginning at age 45, and for persons with a life expectancy of at least 10 years.
According to the American Cancer Society, individuals aged 50 or more should talk to a health care provider about the pros and cons of PSA testing so that they can make an informed decision regarding testing. STI screening needs based on risk. Syphilis cases are on the rise.

Inspect and palpate

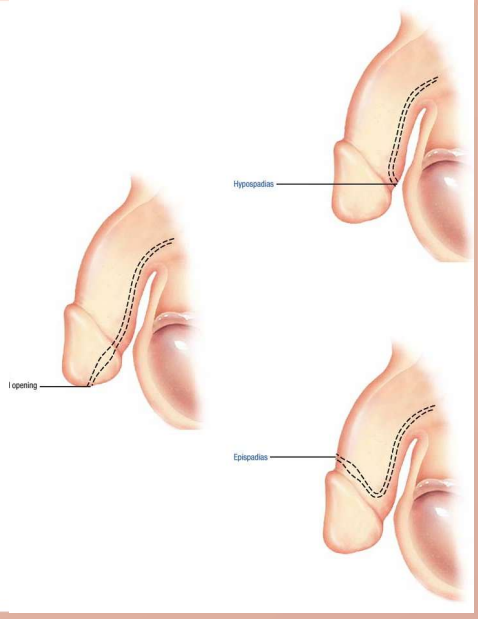
- Penis
  - Base
    - Free of lice & lesions
  - Shaft
    - Soft, flaccid, nontender
  - Foreskin
    - Intact & uniform in color
  - Glans
    - Phimosis & paraphimosis



Inspect the base of the penis and pubic hair for lice
Inspect: the shaft for rashes, lesions or lumps, this may indicate STI or cancer.
Inspect the foreskin: observe for color, location and integrity of the foreskin in uncircumcised men.
Inspect the glans. Chancres (red, oval, ulcerations) from syphilis, genital warts, and pimple-like lesions from herpes are sometimes detected on the glans
Foreskin: foreskin should retract easily. Chancres (red, oval ulcerations) from syphilis, genital warts, and pimplelike lesions from herpes are sometimes detected on the glans.
Abnormal: Phimosis: A tight foreskin that cannot be retracted
Paraphimosis: A foreskin that once retracted cannot be returned to cover the glans

Inspection of urinary meatus

- Urethral meatus should be located at the tip of the penis
- Hypospadias
  - Opening of the meatus is located on the ventral side of penis
- Epispadias
  - Opening of the meatus is located on the dorsal side of the penis



Note the displacement of the urinary meatus to the ventral surface of the penis in hypospadias and dorsal side in epispadias

- Location of urinary meatus on the glans:
- **Hypospadias** – ventral location of the meatus on the underside of the penis
- **Epispadias** – dorsal location of the meatus on the top of the penis

Inspect and palpate

- Inspect the scrotum
  - Should hang at or below the level of the penis
- Inspect the scrotal skin
  - Thin and rugated with little hair, sebaceous cysts are a normal finding
- Palpate the scrotal contents
  - Use thumb and 2 fingers to gently palpate testes

Scrotum: left should hang a little lower than the right side: Enlarged scrotal sac may result from fluid (hydrocele), blood (hematocele), bowel (hernia) or tumor (cancer). A varicocele is an enlargement of the veins within the scrotum which may cause low sperm production and decreased sperm quality

Skin: should be free from lesions
Palpate the scrotal contents: **using your thumb and first two fingers**, note the size, shape, consistency, nodules, masses and tenderness, use gentle pressure. Testes are smooth, firm, rubbery, mobile, free of nodules, and rather tender to pressure. An absence of testes suggests an undescended testicle. Tenderness and swelling may indicate acute orchitis, torsion, hernia, or epididymitis

- Scrotal swelling is taut and pitting – occurs with heart failure, renal failure and inflammation
- **Inguinal lymph nodes:** small, soft, discrete and movable is normal

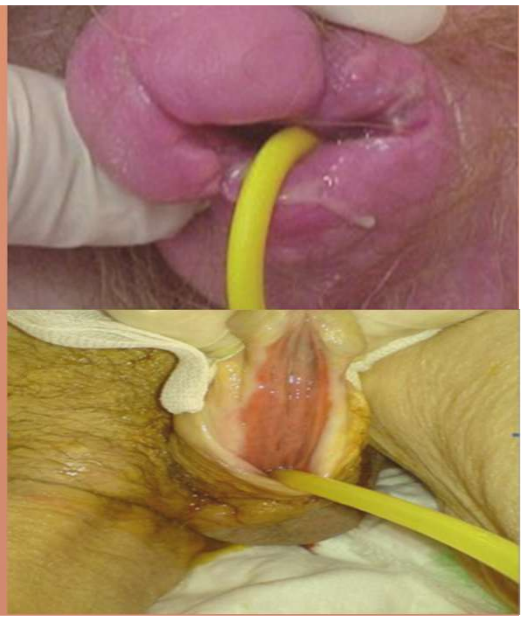
If a mass is found during examination: have the client lie down, note if the mass disappears, if it remains, auscultate it for bowel sounds, and gently palpate the mass and try to push it upward into the abdomen

Inspection of anus and rectum

- Inspect the perianal area
- Use the left lateral position
  - Buttocks & anal opening
    - Lumps, ulcers, lesions, rashes, redness, fissures, thickening of the epithelium
- Abnormal findings may indicate
  - STI, cancer, hemorrhoids, mass or abscess; thickening of the epithelium suggests repeated trauma from anal intercourse

The anal opening should appear hairless, moist, and tightly closed. The skin around the anal opening is coarser and more darkly pigmented. The surrounding perianal area should be free of redness, lumps, ulcers, lesions, and rashes.

Inspect insertion site of foley catheter

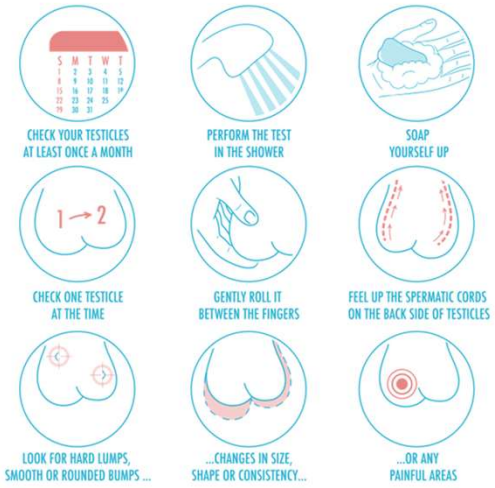


If you are working with a client that has a chronic foley catheter, what do I mean by a chronic foley catheter? … You need to inspect the penis. WHY? Drainage from infection, check the tubing for erosion.

Reasons for a chronic foley: neurogenic bladder, spinal cord injuries, multiple sclerosis, Parkinson's disease

Testicular self-exam (TSE)

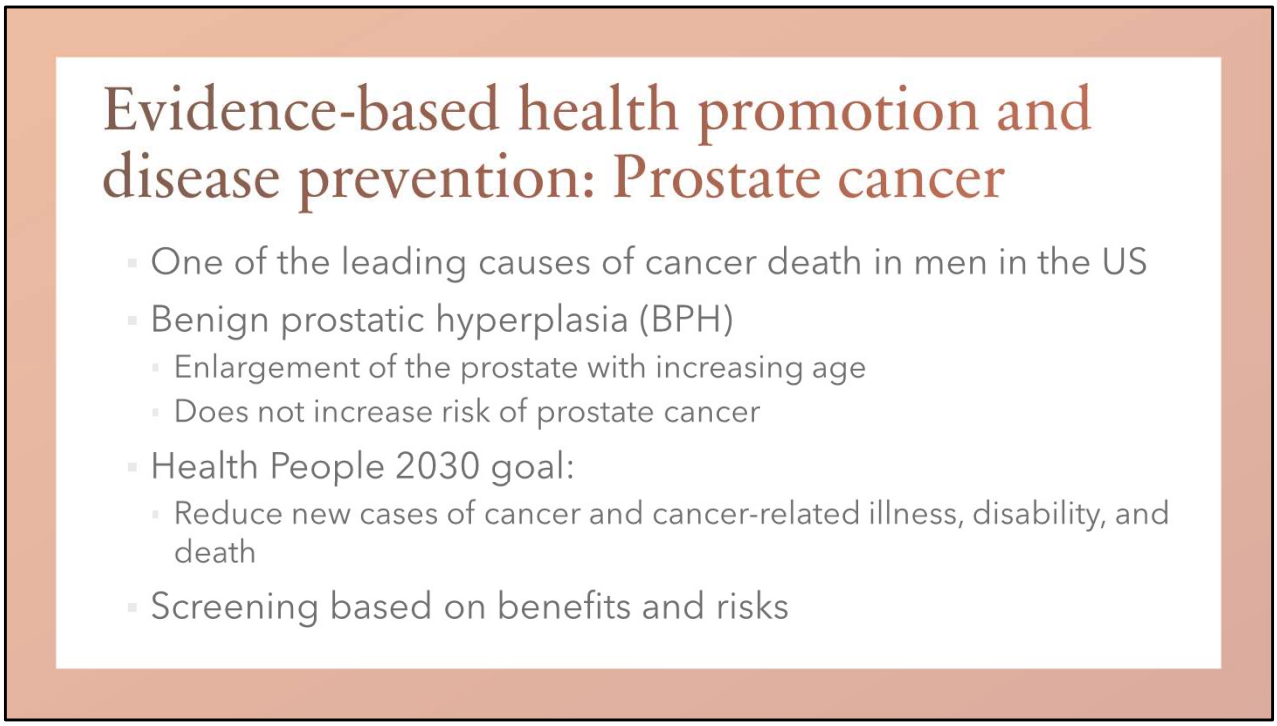
- Begin at age 13-14
- Once a month, in the shower
  - Check for changes which need to be reported immediately
- Surface should feel rubbery and smooth
- Sebaceous cysts are a normal finding
  - Non-tender, 1cm nodules



EDUCATION: TSE should be performed once a month. Use both hands to palpate the testis; the normal testicle is smooth and uniform in consistency. With the index finger and middle fingers under the testis and the thumb on top, roll the testis gently in a horizontal plane between the thumb and fingers. Feel for any evidence of a small lump or abnormality. Follow the same procedure and palpate upward along the testis. Palpate up the spermatic cord on the back of the testis. If you find any evidence of a small pea-like lump consult your physician. It could be due to an infection or a tumor.

Older adult considerations

Thinning hair/decreases	Graying pubic hair	Decreased size of penis	Testicles decrease in size and feel less firm
Scrotal sac is pendulous with less rugae	Scrotal sac becomes excoriated if continuously sat on	Production of sperm decrease around age 40, but continues until age 80-90	After age 55-60 testosterone production declines slowly



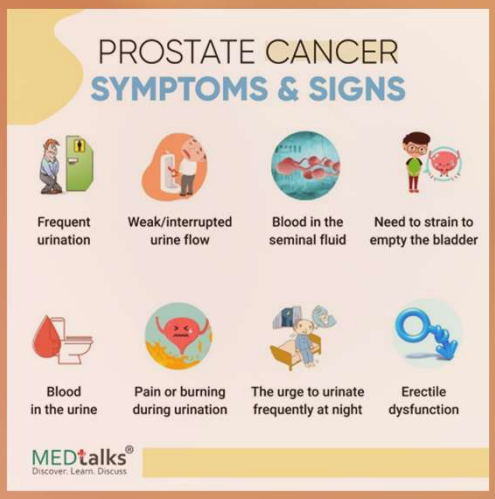
According to the ACS, prostate cancer is one of the leading causes of cancer death in men in the United States, second only to lung cancer. About 1 in 8 men in the United States will get prostate cancer, and 1 in 44 of them will die. Prostate cancer is slow growing and can be readily treated if found early. There is no sure way to prevent prostate cancer, but diet and lifestyle behaviors are thought to help with prevention.

A common problem in almost all men as they grow older is an enlarged prostate (benign prostatic hyperplasia). Some symptoms of benign prostatic hyperplasia and prostate cancer are the same. Having benign prostatic hyperplasia does not raise your risk of prostate cancer.

The ACS concluded that the decision to screen for prostate cancer should be an individual one, based on discussion of benefits and harms with their health care provider. There is NO routine standard screening test. Digital exam can be done to palpate the prostate. The PSA (prostate specific antigen) blood test is available, but not perfect. Levels may be elevated in clients without prostate cancer and may be within normal range for those that do.

Prostate cancer

- **Risks**
- **Client education**
- **Symptoms:**
  - **Trouble urinating**
  - **Decreased force of urine stream**
  - **Blood in urine or semen**
  - **Discomfort in pelvis**
  - **Bone pain**
  - **Erectile dysfunction**
  - **Unexplained weight loss**



Risks

Age >50, African American or Caribbean males of African origin, family history, excessive alcohol intake

Client education

Prostate cancer has no proven method of prevention, but there are some lifestyle changes that may lower the risk. Research indicates that men with excess body weight might be more prone to advanced and fatal prostate cancer.

To potentially reduce the risk, it is advisable to maintain a healthy weight, engage in regular physical activity, and adopt a balanced diet rich in colorful fruits, vegetables, and whole grains, while limiting red and processed meats, sugar-sweetened beverages, and highly processed foods.